جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سامية حسين عبدالله قللو

المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** 17ش محمد عيسى -المطرية - القاهرة

رقم البطاقة : 26001201401644 **رقم التليفون :** 01114417000 **تاريخ تسجيل الإستمارة :** 24-10-2026

العمر(تاريخ الميلاد) : 20-01-1960 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سامية حسين عبدالله قللو
National ID: 26001201401644

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



سامية
حسين عبدالله قللو
١٧ ش محمد عيسى
المطرية - القاهره

٢٦٠٠١٢٠١٤٠١٦٤٤

LB7286946

٢٦٠٠١٢٠١٤٠١٦٤٤ ٢٠٢٥/٠٤

بدون نجل

الثر مسلمة ارملة

البطاقة سارية حتى ٢٠٣٢/٠٤/٠٥

DAR EL SALAM CANCER CENTER
RADIOLOGY DEPARTMENT
ULTRASOUND UNIT

مركز اورام دار السلام
قسم الاشعة التشخيصية
وحدة الموجات الصوتية

Name: sania hussin Abdullah
Date: 9 May 2022

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Mildly enlarged in size (16.8cm at Rt MCL) bright parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic billiary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size . homogenous.*

- **Kidneys: -**
  - *Both are average in size,shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Empty*

Uterus and adenexeal *: no abnormalities could be seen*

Opinion:

Mild hepatomegaly with Bright texture likely fatty
Marked colonic gaseous distention

Best regards

Dr/ncumeen Ramadan MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Name: سامیه حسین عبدالله قللو	
MR. : 6009	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a history of metastatic breast cancer to bone from 6/2021 through 2023 and has continued aromatase inhibitor therapy with aromasin. Imaging with CT and bone scan was requested to assess disease status.

Aromasin was administered today and will be continued through the next month at the pharmacy until radiology. Zometa is due on 02/05/2026, and aromazine will be given in day-care pharmacy after labs, CT, and bone scan.

Post-metastatic breast cancer to bone was noted with ER-positive, HER2-negative status. The patient received taxol and arimidex until 2021, then began bi-weekly afatinib and aromasin for two years. CT scans in March 2026 showed bilateral iliac sclerotic foci; bone scan on 20/4/2026 demonstrated sclerosis of old osseous lesions in the left deltoid/shoulder region. New uptake was seen in the left 7th rib, likely traumatic, and a radiologic correlation was planned. The patient reported a headache; MRI brain was scheduled after three months of follow-up.

Clinical pharmacy noted that Zometa was administered today and labs were within normal limits.

An MRI of the brain performed in May 2026 showed no evidence of brain metastasis. The patient continues aromasin as a single agent with CT follow-up every three months, with the next visit scheduled for 4/7/2026 at the pharmacy.

The patient remains on aromasin for metastatic breast cancer to bone with supportive care, with the next visit planned for 13/10/2026.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

Patient Name : Samia Hussein Abdallah **Date: 13/12/2022**
Referring Physician: Prof. Dr.

Whole body PET/CT

History and clinical data:-
62 years old female patient with right MRM
Radiopharmaceutical:
F-18-FDG mCi injected I.V.
Fasting blood glucose:
mg/dl
Technique:-
Approximately one hour following the IV injection of the radiopharmaceutical, a total body PET – CT was acquired from mid thigh to base of skull with the arms extended above the head. CT was acquired for the purpose of anatomic localization & attenuation correction. Data was processed & displayed according to departmental protocol.

Report:

Hepatic Reference: max SUV ~3.4

Head & Neck:

- *Bilateral low grade FDG avid upper deep cervical lymphadenopathies, the left sided IIb . It (Reaches up to 0.9cm) with max. SUV~2.1.*
- *The brain exhibited normal FDG bio-distribution.*
- *Physiologic FDG uptake is seen in the oropharynx, salivary glands & larynx.*
- *No focal diffuse thyroid gland FDG uptake could be elicited.*

Breast & Axilla:

- *Evidence of right MRM with clear operative bed, no metabolically active FDG avid local tumoral recurrence / residue.*
- *Low grade FDG avid bilateral axillary multiple nodular masses the largest is noted at right sided, measures (2.7x3cm) with max. SUV~3.3).*

Chest:

- *Bilateral pulmonary lower lobes atelectatic changes.*
- *No metabolically active FDG avid mediastinal or hilar lymph nodes are seen.*
- *There are no pleural or pericardial abnormalities.*

PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name : Samia Hussein Abdallah **Date: 13/12/2022**
Referring Physician: Prof. Dr.

- *Physiologic FDG uptake is noted in the myocardium.*

Abdomen & Pelvis:

- *Hepatomegaly with normal FDG uptake & no metabolically active FDG avid focal lesions.*
- *The spleen is average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesion.*
- *Low grade FDG avid abdominal left para aortic, superior mesenteric lymphadenopathies (Subcentimetric) with max. SUV~2.1.*
- *Physiologic FDG excretion is seen in the kidneys & bladder.*

Musculoskeletal:

- *Multiple disseminated combined lytic & sclerotic osseous lesions electing no FDG uptake except left scapular, (Low grade FDG uptake with max. SUV~3.8) consistent with healing process.*
- *Spondylo-degenerative changes are seen at D-L*
- *Normal FDG activity is seen along the rest of the surveyed skeleton. No sclerotic or lytic lesions are noted on CT.*

PET Scan
[[DOCTYPE:PET_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Total DLP (mGy.cm):

Patient Name: Samia Hussein Abd Allah	Date : 04/05/2024
Exam : C.T CHEST	Age : 64 Year(s)
Medical ID: 3682/2019	

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS: -

** Known case of breast cancer as compared to 26-08-2023

Chest:

- Evidence of right mastectomy with no gross operative bed soft tissue masses.
- Still noted prominent right axillary lymph nodes, the largest 18x12 mm
- **Stationary course** of the fore noted few bilateral parenchymal and fissure based millimetric pulmonary nodules.
- **Still noted:**
  - Right upper and middle lobar, anterior subpleural post-radiotherapy changes.
  - Lingular atelectatic bands.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Borderline cardiac size.
- Left sided porta cath is seen
- Small hiatus hernia

Abdomen:

- The liver is enlarged in size with homogenous CT texture. No focal lesions on noncontrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

CT Scan
[[DOCTYPE:CT_SCAN]]

وزارة الصحة والسكان
المراكز الطبية المتخصصة
مركز أورام دار السلام
هرمل، سابقا

Name :
Age :
No.:

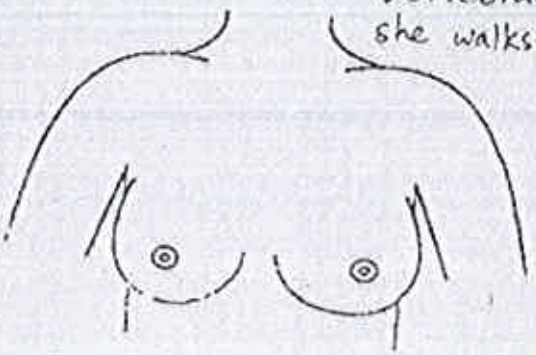
٢٠٢١/٦/٥

MDT FORM

Clinical data :

61 years
PS 2T
→ Rt MRM 10/2017
IDC grade II
(ER+ve 9/15
SM-ve
GR+ve 3/3
Ki-67 <20%

OIE → bone tenderness over dorsolumbar vertebrae.
she walks support



Reason of discussion:

MDT Outcome and Plan

Signature :

Patient's Name:	**Samia Hussein Abdallah Qallu**
Date:	**2026-08-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Post Contrast Multi-Slice CT scan of the Chest, Abdomen and pelvis Revealed

A Known case of Rt breast cancer underwent MRM on follow up.

Chest findings

- ***Small right sided fissural based nodule .... Likely insignificant***
- ***Bilateral basal atelectatic bands.***
- *No detectable suspicious nodules.*
- *Patent trachea, main and segmental bronchi*
- *Normal both **hilar regions**. No hilar masses or lymphadenopathy.*
- *Clear **pleural surfaces** with no evidence of pleural effusion.*
- *Normal **cardiac** size, no pericardial effusion.*

Abdomen and pelvis findings

- *Mild enlarged **liver**. No definite enhanced focal lesions .*
- *No dilated intra-hepatic **biliary radicles**.*
- *Normal caliber patent **Portal Vein***
- *Normal **gallbladder**. No hyperdense gall stones. Normal extrahepatic bile ducts.*
- *Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.*
- *Normal size, shape and density of the **spleen**. No focal lesions.*
- *Normal size, and contrast excretion by **both kidneys**. Normal parenchymal thickness. No renal, masses stones or back pressure changes.*
- *Normal both **supra-renal glands**, no masses.*
- *No **ascites.***
- *No significant abdominal or pelvic **lymphadenopathy***
- *Normal **aorta, I.V.C.***
- *Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.*
- *No **pelvic collections**.*
- ***Degenerative changes of the scanned bones**, No aggressive bony lesions.*
- ***First degree spondylolisthesis of L5 over S1 vertebrae with bilateral fracture pars.***

CONCLUSION:

- ***Stationary course.***
-

Ibrahim

Name : Samia Hussein Abdallah Qallu	**Date : 06-10-2026 09:20**
Branch : Maadi	**Medical No. : 477662**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

NILE DIAGNOSTIC CENTER

Tc-99m MDP WHOLE-BODY BONE SCINTIGRAPHY REPORT

Clinical Indication: Staging and follow-up for skeletal metastatic disease in a 66-year-old female with breast carcinoma since 2022, post-surgical excision, chemotherapy, and radiotherapy, presenting with generalized bone pain.

Procedure: Whole-body anterior and posterior planar bone scintigrams were acquired following intravenous Tc-99m MDP administration.

FINDINGS

- Non-uniform mild tracer uptake is seen along the spinal column, consistent with age-related degenerative spondylosis, without focal intensely avid osteoblastic lesions.
- A focal hyperconcentration is noted in the anterior left rib cage, likely benign and possibly post-traumatic, with symmetrical physiological uptake throughout the remaining ribs and sternum.
- Physiological distribution is noted across the calvarium and pelvic girdle.
- Increased uptake is seen at both knees, consistent with degenerative osteoarthritis, with mild focal activity at the tarsal joints bilaterally and symmetrical mild distribution in both shoulder and wrist joints.
- No focal diaphyseal cortical lesions are seen in the long bones.
- Physiological renal uptake with normal bladder accumulation.

IMPRESSION

- No definitive scintigraphic evidence of active osteoblastic skeletal metastatic disease.
- Isolated focal uptake in the anterior left rib cage, most likely traumatic, for radiological correlation if clinically indicated.

Dr. Alaa Nagi, MD Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]